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February 16, 2026

COMMITTEE SUBSTITUTE
FOR

SENATE BILL NO. 1566

By: Stanley of the Senate

and

Stinson of the House

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[ Medicaid - behavior analysis treatment - medical
necessity criteria - requirements - telehealth -
effective date ]
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BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

SECTION 1. AMENDATORY 56 O.S. 2021, Section 1011.12, is amended to read as follows:

Section 1011.12. A. The Oklahoma Health Care Authority, in conjunction with the Department of Mental Health and Substance Abuse Services, the State Department of Health and the State Department of Education shall examine the feasibility of a state plan amendment to the ~~Oklahoma Medicaid Program~~ state Medicaid program for applied behavior analysis treatment of autism spectrum ~~disorders~~ disorder.

B. On or before December 31, 2016, the Authority and partnering agencies shall submit a report to the President Pro Tempore of the Senate, the Speaker of the House of Representatives and the Governor

1 estimating the potential costs to the state, clinical findings,
2 reviews of pilot projects and research from other states on the
3 effects of applied behavioral analysis treatment on autism spectrum
4 ~~disorders~~ disorder.

5 C. Beginning July 1, 2017, and subject to the availability of
6 funding, the Authority and partnering agencies shall draft a state
7 plan amendment for applied behavior analysis treatment of autism
8 spectrum ~~disorders~~ disorder. The provisions of this subsection
9 shall only apply if the report required by subsection B of this
10 section demonstrates applied behavioral analysis treatment to be
11 evidence-based and essential to qualifying participants in the
12 ~~Oklahoma Medicaid Program~~ state Medicaid program.

13 D. The medical necessity criteria for applied behavior analysis
14 services covered under the state Medicaid program shall include a
15 definitive diagnosis of an autism spectrum disorder by one of the
16 following health care providers, who shall be licensed in this state
17 and who shall be trained in the diagnosis of autism:

- 18 1. Pediatric neurologist or neurologist;
- 19 2. Developmental pediatrician;
- 20 3. Psychologist which may include, but is not limited to, a
21 psychologist working in a school;
- 22 4. Psychiatrist or neuropsychiatrist;
- 23 5. Other licensed physician experienced in the diagnosis and
24 treatment of autism spectrum disorder; or

1 6. Interdisciplinary team composed of a licensed psychologist,
2 physician, physician assistant (PA), or Advanced Practice Registered
3 Nurse (APRN).

4 E. The Authority or a contracted entity shall not impose a
5 general policy that requires all Medicaid members receiving covered
6 applied behavior analysis services to be re-diagnosed with or
7 receive subsequent clinical assessments for autism spectrum disorder
8 as a condition of continuation of coverage after the member has
9 already been diagnosed with autism spectrum disorder by a health
10 care provider as described in subsection D of this section. An
11 initial diagnosis of autism spectrum disorder shall remain accepted
12 indefinitely unless otherwise clinically indicated including, but
13 not limited to, by a significant behavioral or developmental change.
14 Nothing in this subsection shall be construed to prohibit a member
15 or member's parent or legal guardian from requesting a subsequent
16 assessment or a qualified provider from conducting a subsequent
17 assessment if he or she deems it necessary.

18 F. 1. All applied behavior analysis services provided by a
19 contracted provider to a Medicaid member shall be performed by duly
20 qualified professionals as determined by the Authority.

21 2. A registered behavior technician (RBT), board-certified
22 assistant behavior analyst (BCABA), or other professional whose
23 credentials are less than a board-certified behavior analyst (BCBA),
24

1 as determined by the Authority, may only provide applied behavior
2 analysis services to members in person.

3 3. A BCBA may use telehealth to provide applied behavior
4 analysis services to members or to supervise the provision of
5 applied behavior analysis services by a BCABA or other professional
6 described in paragraph 2 of this subsection. The Authority or a
7 contracted entity shall not require a BCBA to be located in this
8 state or within a certain distance of the state border to perform
9 such services or supervision; provided, however, the BCBA shall be
10 licensed in this state under Section 1928 of Title 59 of the
11 Oklahoma Statutes or licensed or credentialed in his or her state of
12 residence.

13 G. As used in this section:

14 1. "Applied behavior analysis" means the design, implementation
15 and evaluation of environmental modifications, using behavioral
16 stimuli and consequences, to produce socially significant
17 improvement in human behavior, including the use of direct
18 observation, measurement and functional analysis of the relationship
19 between environment and behavior;

20 2. "Autism spectrum disorder" means any of the pervasive
21 developmental disorders or autism spectrum disorders as defined by
22 the most recent edition of the Diagnostic and Statistical Manual of
23 Mental Disorders (DSM) or the edition that was in effect at the time
24 of diagnosis;

1 3. "Behavioral health treatment" means counseling and treatment
2 programs, including applied behavior analysis, that are:

3 a. necessary to develop, maintain or restore, to the
4 maximum extent practicable, the functioning of an
5 individual, and

6 b. provided by a board-certified behavior analyst or by a
7 licensed doctoral-level psychologist so long as the
8 services performed are commensurate with the
9 psychologist's university training and supervised
10 experience; and

11 4. "Treatment for autism spectrum disorder" means evidence-
12 based care and related equipment prescribed or ordered for an
13 individual diagnosed with an autism spectrum disorder by a licensed
14 physician or a licensed doctoral-level psychologist who determines
15 the care to be medically necessary, including, but not limited to:

16 a. behavioral health treatment,

17 b. pharmacy care,

18 c. psychiatric care,

19 d. psychological care, and

20 e. therapeutic care.

21 SECTION 2. This act shall become effective November 1, 2026.

22 COMMITTEE REPORT BY: COMMITTEE ON HEALTH AND HUMAN SERVICES
23 February 16, 2026 - DO PASS AS AMENDED BY CS
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